جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : دعاء حسن احمد عبدالسلام
المحافظة : الجيزه **الحي / المدينة :** كرداسه **العنوان :** 0
رقم البطاقة : 28105202102206 **رقم التليفون :** 01154439985 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 20-05-1981 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: دعاء حسن احمد عبدالسلام
National ID: 28105202102206

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



دعاء
حسن احمد عبدالسلام
١٩٧ ش ناهيا-المعتمديه
مركز كرداسة - الجيزه

٢٨١ ٠٥ ٢٠ ٢١ ٠٢٢ ٠٦

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حاصلة على دبلوم تجارة

انثى مسلمة متزوجة
احمد حسن احمد عبدالمجيد
البطاقة سارية حتى ٢٠٣٢/٠٥/٠٧ ١

الهيئة العامة للتامين الصحى
الادارة المركزية لتكنولوجيا المعلومات
ودعم القرار

HEALTH INSURANCE ORGANIZATION
CENTER DEPARTMENT OF
INFORMATION AND DECHON SUPPORT

تاريخ الاصدار :2023-3-27

جهه الاصدار :فرع الجيزة

الى من يهمه الامر .....

الهيئه العامة للتامين الصحى تعمل من خلال قوانين منظمة للانتفاع الخاضعين لقانون التامين الاجتماعى رقم 79 لسنه 1975 المعدل بالقانون 25 لسنه 1977 "القوى العامله والمعاشات والارامل " و كذلك قانون 99 لسنه 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنه 2012 " الاطفال دون السن المدرسى" وقانون رقم 23 لسنه 2012 الخاص بالمرأة المعيله .

وطبقا لقاعدة البيانات المرسلة من التأمينات الاجتماعيه

وطبقا لقاعدة البيانات المرسلة من الضمان الاجتماعى

والبحث تبين أن

السيد (ة) دعاء حسن احمد عبد السلام بطاقة رقم 28105202102206

وطبقا الى خطاب من التأمينات الإجتماعية بفرد عدم الأنتفاع

يعتبر من الفئات غير المنتفعه بالتامين الصحى صدر هذا الخطاب دون اى مسئولية على الهيئة العامة للتأمين الصحى حال خضوع المواطن لقانون التامين الصحى الشامل

وشكرا

تاريخ الطباعه :2023-3-27

الموظف المختص

امتثال نعيم

إفادة مركز نظم المعلومات

يعتمد،،،

مسلسل 584730

الخطاب ساري لمدة ثلاثة اشهر من تاريخ الإصدار إلا فى حاله تغير حاله المواطن من قبل الهيئة القومية للتأمينات الاجتماعية أو وزارة التضامن الاجتماعى أو وزارة التربية و التعليم

الادارة المركزية لتكنولوجيا المعلومات و دعم القرار

Name: دعاء حسن احمد **MR. :** 324	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

The patient is a 45-year-old individual who underwent luminal neoadjuvant chemotherapy (NACT) followed by a right lumpectomy and sentinel lymph node biopsy in October 2023. Post-operative management included a radiation therapy course (PORT) and adjuvant endocrine therapy with Zoladex (goserelin) and Femara (letrozole), with the last Zoladex injection administered on 14-January-2026.

On 8-April-2026 the patient presented for a Zoladex injection and Femara continuation; the next Zoladex dose was scheduled for 1-July-2026, with a planned three-month course of Femara to be managed by pharmacy.

On 1-July-2026 an annual assessment was conducted, during which a Zoladex injection was given and Femara was continued for an additional three months, with pharmacy coordination for ongoing supply.

On 13-August-2026 the patient received a Zometa (zoledronic acid) infusion; laboratory results were accepted and the patient was noted to have a BBSM (bone-specific marker) value of 13-August-2026. No additional complications or toxicities were documented at this time.

تحريرا في ,

التاريخ :	**20-09-2026**	**الوقت :**	

الطبيب المعالج : اسماء محمد مصطفي نصر

يعتمد ,

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:

1- Name : دعاء حسن احمد عبدالسلام | Sex: | 3- Medical number: Female | 83129564 | Old Med : | Old Med : 804-202

2-ID number: 28105202102206 | Patient Age : 43 Y

Family history	Family History -ve	Phone	Phone 01154439981	BMI	BMI	REASON O	
marital status	Marital Status MARRIED	NRS Score	NRS Score	GROUP	GROUP C	Date of MDT	
Co morbidity	Co Morbidity	Menopausal status	Menopausal Status PREMENOPAUSAL	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :

Mets Workup :

BX	ER	PR	Her 2 Neu	SISH	Ki67
BX COMMENT:	8/8 ER COMMENT:	6/8 PR COMMENT:	0 Her 2 Neu COMMENT:	SISH COMMENT :	40

T	N	M
2	2	0

Sonomammography

RT LOQ mass 8-9 o'clock 2.5*2 cm

PATIENT HISTORY

pt received NACT AC x4 ++taxotere x 4 rt breast conservative + axillary LN dissection: (ypT2, N1)
no remnant of invasive component remnant of ductal carcinoma insitu
free margins including extra inferior & deep margins LN +VE 1/13
partial pathological response YPTis YPN1

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

for PORT, zoladex/AI for 5 ys
consider extended , zometa/6 mn for 3 ys after dental

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/ هاني فكري	RADIOLOGY	RADIOLOGY د/ هبه فرج
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Patient Name	: الأستاذه / دعاء حسن احمد عبدالسلام	Request No.	: 13007498
Patient No.	: 28105202102206	Request Date	: 06/03/2023
Path No.	: 707/2023	Verification Date	: 22/03/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 41 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical data:
Right breast mass with enlarged right axillary lymph nodes

ERM036727129

Nature of specimen:
Tru cut biopsy, collected at 6/3/2023 , 2:45 pm

Gross examination
1)Right breast lesion:
Multiple greyish white tissue cores up to 1.5 cm , totally submitted.
2)right axillary lymph nodes:
Multiple greyish white tissue cores up to 1cm , totally submitted.
Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)
Microscopic examination:
* Histological type: Invasive duct carcinoma .
* Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+1) Grade II
* DCIS: Focal
*Angioinvasion: Could not be detected within the limits of biopsy recieved
2)Right axillary lymph node:
* Histological type:Metastatic adenocarcinoma compatible with mammry origin
*IPT:
ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)
PR: Strong nuclear reaction in 30% of tumor cells (Allred score 6/8)
Her-2: Weak membrane reaction in 10% of tumor cells (score 1+)
Ki 67: 40 % nuclear reaction in tumor cells
Positive control showed appropriate reactivity

Verified by :
dr.esraa magdy

Room No. (4) - First Floor Building No. 297- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذه / دعاء حسن احمد عبدالسلام	Request No.	: 13007498
Patient No.	: 28105202102206	Request Date	: 06/03/2023
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Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 41 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Diagnosis:
Right breast mass with enlarged right axillary lymph nodes, tru cut biopsy:
1)Right breast:
Invasive duct carcinoma, Grade II .
2)Right axillary lymph nodes:
Metastatic adenocarcinoma compatible with mammry origin
IPT:
ER: Positive (score 8/8).
PR: Positive (score 6/8).
HER-2: (score 1+).
Ki67: Positive in 40% of tumor cells.

Prof.Gamal Abdelatty
Dr.Marwa Mahmoud

Verified by :
dr.esraa magdy

قرية رقم (2) - الدور الأول - مبنى رقم (287) - كورنيش النيل - بجوار كلية ... - القاهرة - مصر
Room No. 045 - First Floor Building No. 287- Nile Corniche - Sitar El Qudessa- Cairo, Egypt

Patient Name	: الأستاذه / دعاء حسن احمد عبدالسلام	Request No.	: 13008593
Patient No.	: 28105202102206	Request Date	: 28/10/2023
Path No.	: 3723/23	Verification Date	: 11/11/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 42 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer, post NACT.
Natura Specimen: Right breast conservative surgery and axillary lymph nodes.

Gross Examination:
* Procedure: Right breast conservative surgery .
Fibrofatty tissue piece measured 5.5x4.5x2 cm with covering skin ellipse measured 3.5x0.7 cm.
* Tumor extent: A wire guided greyish white tumor bed site
* Size: 1.9x1.5 cm0.1 cm from inferior margin .
* Another greyish white area measured 2.5x2 cm was located near lateral margin .
* Extra inferior margin : 5x4 cm
* Extra deep margin : 5x4 cm
* Axilla measured 9x7 cm entangling 13 lymph node up to 2x1.5 cm .

Microscopic Examination:
* No remnants of invasive component .
* Ductal carcinoma in situ; 5% remnants .
* Resection margins: Free margins (including the extra inferior and extra deep margin)
* Histologic response to therapy: Partial pathological response .
* Lymph nodes : Positive in 1/13 .
* Frozen section : Inferior and deep margins showed atypical crushed cells .

Diagnosis:
Right breast conservative surgery and axillary lymph nodes, post NACT:
* No remnants of invasive component .
* Remnants of ductal carcinoma in situ .
- Free margins including extra inferior and deep margin .
- Lymph nodes : Positive in 1/13 .

Verified by :
Rana fathy

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذه / دعاء حسن احمد عبدالسلام	Request No.	: 13008593
Patient No.	: 28105202102206	Request Date	: 28/10/2023
Path No.	: 3723/23	Verification Date	: 11/11/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 42 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

- Partial pathological response .
- Stage: ypTis, ypN1
- RCB: 1.64, RCB class II

Signature
Dr. Rana Fathy
Dr. Menan Hossam

Verified by :
Rana fathy

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

GUSTAVE ROUSSY INTERNATIONAL

Patient Name: Doaa Hassan Ahmed Abd Al Salam	Date : 24/08/2025
Exam : MAMMOGRAPHY BOTH	Age : 44 Year(s)
Medical ID: 804/2023	Pat. ID: 31907

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Post right CBS and ALAND followed by radiotherapy, routine follow up study.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- UOQ architectural distortion with no speculations, masses, asymmetry , or suspicious microcalcifications.
- Thickened skin and coarsen trabeculae.
- Ultrasound demonstrated UOQ, area of parenchymal disruption underneath the operative scar with no solid , cystic lesions or fluid collections.
- Thickened skin reaching 7 mm and coarsen trabeculae.
- Normal nipple/areola complex.
- Clear right axilla. No pathological infra or supra-clavicular lymph nodes

Left breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses , distortion, asymmetry , or suspicious microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions .
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 2): Post-therapeutic sequalae

Left breast (BIRADS 1):. Normal study.

Recommendation:-

For annual follow up.

Best Regards,

DR, Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routing screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	
BIRADS 6: Pathologically proven malignant	98-100%

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Doaa Hassan Ahmed Abd Al Salam	Date : 31/08/2025
Exam : X-RAY CHEST	Age : 44 Year(s)
Medical ID: 804/2023	Pat. ID: 31907

Plain X-ray **Chest** (PA view) revealed:

- *Clear both lung fields.*
- *Clear both costo-phrenic angles.*
- *Normal cardiac size & configuration.*
- *Normal thoracic cage*

Signature Dr . Nermien Ramadan

31/08/2025 12:54 PM

Name: Doaa Hassan

Date: 10/5/2023

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: Right breast cancer pathologically proved to be IDC

TECHNIQUE: CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: mild background enhancement

ACR: b (scattered fibro-glandular densities)

RIGHT BREAST:

- **Low energy (Non contrast) images**: show mid outer irregular hyperdense mass lesion with indistinct margin and intra-lesional punctate microcalcifications.
- **Recombined (Post contrast) images**:
  - ☐ Corresponding heterogeneous moderate mass enhancement is noted, measuring 2.8 x 1.6 x 2.3 cm along its maximum dimensions, being at about 4 cm from the nipple.
  - ☐ Adjacent indeterminate faintly enhancing nodules are noted (on CC view) occupying an area of about 7.8 cm, being at about 1.6 cm from the index lesion.
  - ☐ The previously reported deeply seated central asymmetry shows no notable enhancement (noted in MLO view).
- Normal skin / nipple / areolar complex.
- **Complementary US**: large pathologically enlarged lymph node is noted showing infiltrated fatty hilum. No evidence of pathological supra or infraclavicular lymph nodes.

LEFT BREAST:

- **Low energy (Non contrast) images** No masses, asymmetry or distortion or suspicious calcifications.
- **Recombined (Post contrast) images** show, no pathological uptake.
- Normal skin / nipple / areolar complex.

IMPRESSION:

RIGHT BREAST:

- Mid outer malignant mass associated with adjacent indeterminate faintly enhancing nodules and ipsilateral pathologically enlarged level I axillary lymph node (BIRADS 6= pathologically proven malignancy).
- Right breast deeply seated asymmetry with no notable enhancement (BIRADS 3), for 6 months follow up by single view mammography

LEFT BREAST: No pathologically enhancing lesions (BIRADS 1= normal)

Best regards,

Dr. Sherihan Fakhry, MD

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Doaa Hassan Ahmed Abd Al Salam	Date : 28/03/2023
Exam : C.T CHEST WITH CONTRAST	Age : 42 Year(s)
Medical ID:	Pat. ID: 31907

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Reported case of right breast cancer

1. ***Chest:***

- Right breast LOQ irregular speculated mass lesion measuring 2.5 X 1.5 cm.
- Right axillary level I and II pathologically enlarged lymph nodes the largest measures 4 X 3 cm.
- no sizable pulmonary nodules.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. ***Abdomen and pelvis:***

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- IUD seen in place.
- Right adnexal cystic lesion mostly functional.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. ***The scanned skeleton:***

PATIENT NAME: **DOAA HASSAN AHMED ABDEL SALAM** GENDER: **Female** AGE: **41 y**	PATIENT ID #: **2023221552** ORDER #: **2196186,2196187** DATE OF SERVICE: **05-Mar-2023 09:39 AM**

Report: - all studies

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Right breast palpable lump.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Scattered fibroglandular densities (ACR B).

RIGHT BREAST:

- Right LOQ dense lesion with indistinct margins with overlying loosely attached microcalcifications.
- Other few scattered microcalcific foci.
- The US revealed right 8-9 o'clock irregular hypoechoic lesion with internal vascularity on color doppler measuring about 25 x 18 mm . Its seen touching the under-surface of the skin and located at about 4 cm from the nipple.
- Right LOQ focal deeply seated focal assymetry corresponds to subtle parenchymal distortion seen deeply seated at 9 o'clock.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Right axillary (level I) pathologically enlarged lymph node measuring about 48.5 x 25.5 mm showing lost hilum most likley amalgamated nodes.
- No infra or supraclavicular pathological lymph nodes.

LEFT BREAST:

- No spiculated mass lesions or grouped microcalcifications.
- This was confirmed by Complementary Ultrasound which showed no solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.

NAME: Doaa Hassan Ahmed
DATE: 26.10.2023

MAMMOGRAPHIC GUIDED STEREOTACTIC WIRE LOCALIZATION:

CLINICAL SUMMARY:

Right breast CA that was under NAC, for localization of the LOQ previously inserted clip

TECHNIQUE:

Under complete sterile conditions, 3ml of local anaesthetic were administered at the sites of concern in the right breast.
A 21G wire was used to localize the LOQ clip located at 8-9 o'clock, about 5cm away from the nipple, and the wire was inserted 2cm deep with the hook about 2mm from the clip.

Skin marking was also performed.

Post procedure films were done in MLO and CC views to confirm.

Best regards,
Dr. Mahi Mokhtar

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Doaa Hassan Ahmed Abd Al Salam	Date : 04/09/2024
Exam : MAMMOGRAPHY BOTH	Age : 43 Year(s)
Medical ID: 804/2023	Pat. ID: 31907

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Patient had CBS in right breast.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B).
- No left breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Right breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Left breast benign finding (BIRADS 2)

Signature Dr. Nehal Hamdy

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